

Talazoparib (Talzenna®) and enzalutamide

Talazoparib and enzalutamide are cancer drugs. They are used together to treat advanced prostate cancer.

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What is talazoparib (Talzenna®) and enzalutamide?

Talazoparib and enzalutamide is a combination of cancer drugs. They are used together to treat prostate cancer that has [spread to other parts of the body](#). You may have it if other hormone therapies have stopped working. This is called hormone-relapsed metastatic prostate cancer.

It is best to read this information with our general information about [cancer drug treatments](#) and the [type of cancer](#) you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How talazoparib and enzalutamide work

[Talazoparib](#) is also called Talzenna[®]. It is a type of [targeted therapy drugs](#) called a PARP inhibitor.

PARPs are proteins that help damaged cells to repair themselves. Talazoparib blocks (inhibits) how PARP proteins work. Without PARP proteins, cancer cells may become too damaged to survive and then die. This can help control the cancer.

Enzalutamide is a hormonal therapy drug. It is used to treat prostate cancer.

Hormones are chemicals that our bodies make. They act as messengers and help control how cells and organs work. Hormonal therapy drugs change how hormones are made or work in the body.

Prostate cancer cells need a hormone called testosterone to grow. Testosterone is mainly made by the testicles. Enzalutamide blocks testosterone from reaching the prostate cancer cells. This may slow down the growth of the cancer cells.

Having talazoparib and enzalutamide

You have talazoparib as capsules and enzalutamide as tablets you can take at home. You usually take this treatment with your existing hormonal therapy.

During your treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

During your course of treatment, you will have regular blood tests. This is to check that it is safe for you to have treatment.

You usually have talazoparib and enzalutamide for as long as it is working for you and any side effects can be managed. Do not stop taking this treatment without talking to your doctor.

Taking talazoparib capsules and enzalutamide tablets

Your cancer team will discuss your treatment plan with you. They will give you the capsules and tablets to take home.

Always take these exactly as they tell you to. This is important to make sure they work as well as possible for you. Make sure you:

- swallow them whole with a glass of water
- do not chew, open, break or crush them
- take them at the same time every day.

If you forget to take talazoparib, skip that dose and take the next dose at the usual time. Do not take a double dose.

If you forget to take enzalutamide, take the missed dose as soon as you remember within the same day. If a full day has passed, take the next dose at the usual time. Do not take a double dose.

If you miss more than one dose, you can contact the hospital on the 24-hour number you have been given for advice.

Other things to remember about your capsules or tablets:

- Keep them in the original package and at room temperature, away from moisture, heat and direct sunlight.
- If you are sick (vomit) just after taking them, do not take an extra dose. Take your next dose at the usual time.
- Get a new prescription before you run out. Make sure you have plenty for holidays.
- Do not throw away unused capsules or tablets. Return them to your cancer team at the hospital.
- Keep them safe, where children cannot see or reach them.
- Only handle the tablets yourself if possible. People who are or may become pregnant should not touch the tablets if they are broken or damaged.

Your cancer team may also give you anti-sickness drugs and other medications to take home. Take all your medicines exactly as they tell you to. Do not stop taking any of your medicines unless your doctor tells you to.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Other cancer treatments may cause different side effects. If you are also having other cancer treatment, you may have other side effects.

When you have this treatment, you will have regular appointments with a doctor, nurse or pharmacist. Always tell them about any side effects you have. They can give you:

- drugs to help control most side effects
- advice about managing side effects.

Most side effects can be managed. But sometimes side effects are harder to control. It is important not to stop taking hormonal therapy without telling your doctor. If side effects cannot be managed, your doctor may suggest a different type of hormonal therapy.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Most common side effects

These side effects are likely to be the most common side effects for this combination treatment. They happen to 10 or more people in every 100 people (10% or more) who have the individual drugs.

Risk of infection

This treatment can reduce the number of white blood cells in your blood. These cells fight infection. If your white blood cell count is low, you may be more likely to get an [infection](#). A low white blood cell count is called neutropenia.

An infection can be very serious when the number of white blood cells is low. It is important to get any infection treated as soon as possible. If you have any of the following symptoms, contact the hospital straight away on the 24-hour number:

- a temperature above 37.5°C
- a temperature below 36°C
- you feel unwell, even with a normal temperature
- you have symptoms of an infection.

Symptoms of an infection include:

- feeling shivery and shaking
- a sore throat
- a cough
- breathlessness
- diarrhoea
- needing to pass urine (pee) often, or discomfort when you pass urine.

It is important to follow any specific advice your cancer treatment team gives you.

You will have regular blood tests during treatment. If your white blood cell count is low, your doctor may stop your treatment for a short time, until your cell count increases. Sometimes they will reduce the dose of your treatment.

High blood pressure

Enzalutamide can cause high blood pressure. Tell your doctor or nurse if you have ever had any problems with your blood pressure. You will have regular blood pressure checks when taking this drug. Let your doctor or nurse know if you have any headaches or blurred vision.

Bruising and bleeding

This treatment can reduce the number of platelets in your blood. Platelets are cells that help the blood to clot.

If the number of platelets is low, you may bruise or bleed easily. You may have:

- nosebleeds
- bleeding gums
- heavy periods
- blood in your urine (pee) or stools (poo)
- tiny red, brown or purple spots that may look like a rash – these spots can be harder to see if you have black or brown skin.

If you have any unexplained bruising or bleeding, contact the hospital straight away on the 24-hour number. You may need a drip to give you extra platelets. This is called a platelet transfusion.

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Feeling sick

Your doctor, nurse or pharmacist will prescribe anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to, even if you do not feel sick. It is easier to prevent sickness than to treat it after it has started.

If you feel sick, take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids. If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice. They may change your anti-sickness treatment. Let them know if you still feel sick.

Loss of appetite

This treatment can affect your appetite. Don't worry if you do not eat much for 1 or 2 days. But if your appetite does not come back after a few days, or if you are losing weight, tell your doctor, nurse or pharmacist. They can give you advice. They may give you food or drink supplements. Or they may suggest changes to your diet or eating habits to help.

Tummy pain

You may get pain in your tummy (abdomen), or have indigestion. Your doctor, nurse or pharmacist can give you advice or treatment to help. Contact the hospital straight away on the 24-hour number if your symptoms do not go away, or get worse.

Diarrhoea

This treatment may cause [diarrhoea](#). Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Hot flushes and sweats

Hot flushes are a common side effect of this treatment. During a flush, your neck and face may feel warm. If you have white skin, your skin may look red. If you have black or brown skin, your skin colour may deepen. Flushes can last up to 5 minutes. You might also have sweats or feel anxious or irritable during a flush.

Here are some things you can do to reduce the effects of hot flushes:

- Wear natural fabrics, such as cotton.
- Wear layers so you can remove clothes as needed.
- Use cotton sheets and layers of bedding you can remove.
- Try cooling pads or pillows to keep you cool.
- Keep rooms cool or use a fan.
- Have cold drinks and avoid caffeine and alcohol.

You might have fewer hot flushes as your body adjusts to this treatment. You could try therapies to help you [cope with hot flushes](#), such as:

- [talking therapies](#) like cognitive behaviour therapy (CBT)
- controlled breathing exercises
- yoga
- acupuncture.

If they do not improve, talk to your doctor. Certain drugs can help to improve hot flushes.

Flushes usually stop a few months after treatment ends. But some people continue to have them.

Headaches

This treatment may cause headaches. If you have headaches, tell your doctor, nurse or pharmacist. They can give you advice about painkillers that may help. Tell them if the headache does not get better, or gets worse.

Feeling dizzy

This drug might make you feel dizzy. If you feel dizzy, do not drive or operate machinery.

Bone thinning (osteoporosis)

Taking this treatment for several months or more can increase the risk of [bone thinning](#). This is called osteoporosis. It can make you more likely to get a broken bone (fracture).

You may have bone density scans to check your bone health before and during treatment. These scans can diagnose osteoporosis or osteopenia (low bone density) so that it can be monitored. If your scan shows osteoporosis, your doctor usually prescribes:

- drugs to protect your bones called [bisphosphonates](#)
- calcium and vitamin D supplements.

Regular exercise where you support your body weight, like walking, running and dancing, can help look after your bones. This is called [weight-bearing exercise](#). Other types of strength exercises are also helpful. If you have any bone thinning or the cancer is affecting your bones, get advice from your cancer team first.

Eating a healthy diet and not smoking also helps take care of your bones.

Risk of falls

People taking enzalutamide have an increased risk of falls. You can reduce your risk of falls by:

- keeping the area you walk through clear
- making sure rooms are well lit
- putting a light on if you wake up at night.

If you feel dizzy when walking, you should sit down straight away until it passes. Always tell your doctor or nurse if you have dizziness or feel unsteady when moving around.

Feeling tired (fatigue)

[Feeling tired](#) is a common side effect. Try to pace yourself and plan your day so you have time to rest between activities.

Being physically active can help to manage tiredness and give you more energy. It also:

- helps you sleep better
- reduces stress
- improves your bone health.

If you feel sleepy, do not drive or operate machinery.

Hair thinning

While you are having this treatment, the hair on your head might become thinner, drier and more brittle. This is usually mild.

You might also lose some:

- body hair, such as chest and pubic hair
- facial hair.

If you are worried about this, talk to your nurse about [hair care](#).

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Effects on the heart

This treatment might increase certain risk factors for [heart problems](#). Tell your doctor if you have:

- had any problems with your heart
- high blood pressure
- high levels of cholesterol – a fatty substance in your blood.

They can monitor or treat these if needed. Talk to your doctor if you are worried about this.

Things you can do to take care of your heart include:

- not smoking
- maintaining a healthy weight
- being physically active.

Skin changes

This treatment can [affect your skin](#). It may cause a rash, which may be itchy. Your doctor or nurse can tell you what to expect. If your skin feels dry, try using soap-free cleansers and unperfumed moisturising cream every day.

Always tell your doctor or nurse about any skin changes. They can give you advice and may prescribe creams or medicines to help. Changes to your skin usually improve when treatment ends.

Serious skin changes

Rarely, this treatment can cause a serious skin reaction that needs to be treated immediately in hospital. Contact the hospital straight away on the 24-hour number if you have any of these symptoms:

- a skin rash that is spreading

- blistering or peeling skin
- flu-like symptoms, such as a high temperature and joint pain
- sores on your lips or in your mouth.

Changes to your taste

Some foods may taste different or have no taste. Try different foods to find out what tastes best to you. You may also get a bitter or metallic taste in your mouth. Your doctor, nurse or pharmacist can give you advice. It might help to try:

- sucking sugar-free sour or boiled sweets
- eating cold foods
- eating sharp-tasting fresh fruit.

Taste changes usually get better after treatment ends. We have more information about [coping with changes to taste](#).

Sore mouth and throat

This treatment may cause a [sore mouth and throat](#). You may also get mouth ulcers. This can make you more likely to get a mouth or throat infection. Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals.

Contact the hospital straight away on the 24-hour number, if:

- a sore mouth or throat affects how much you can drink or eat
- your mouth, tongue, throat or lips have any blisters, ulcers or white patches.

They can give you advice, and mouthwash or medicines to help with the pain or to treat any infection. Follow their advice and make sure you:

- drink plenty of fluids
- avoid alcohol and tobacco
- avoid food or drinks that irritate your mouth and throat.

Breast swelling or tenderness

This treatment may cause swelling and tenderness of your breast tissue. This is called gynaecomastia. To prevent this, some people have 1 or more low-dose radiotherapy treatments to the chest before treatment starts. Another type of hormonal drug might be used to treat the breast swelling. Your doctor can give you more advice.

Restless legs

You may find you get restless legs while taking this treatment. This is when you feel you need to move your legs regularly. It is common at night-time. Talk to your doctor or nurse if you get this. They can talk to you about ways to help manage it.

Sexual effects

It is common to lose your [sex drive](#) and have erection difficulties with hormonal therapy. Hormonal therapy may also make your penis shorter and your testicles smaller.

Your doctor can prescribe drugs and treatments to help with erection difficulties. These will not increase your sex drive. But they might work, even with a low sex drive.

Things can improve a few months after you stop taking the drug. But it depends on how long you take hormonal therapy, and what other treatments you have had. Some people continue to have problems after treatment ends.

Mood changes

You may feel low or depressed, or have mood swings, during this treatment. Talking to family and friends about how you feel might help. If mood changes last for more than a few weeks, tell your doctor, nurse or pharmacist. They can talk to you about different ways to manage low mood or [depression](#).

Feeling anxious

This treatment may make you feel anxious. Symptoms of anxiety include:

- not being able to concentrate
- being irritable
- being easily distracted
- feeling restless
- having a constant feeling of dread.

If you feel your anxiety is getting worse, speaking with your healthcare team is the best way to get the help you need.

Memory and concentration

You may notice [changes in your memory](#). You may also find it harder to concentrate.

To help you remember things, try using:

- reminder alarms or notes on your phone
- a diary, calendar or phone app
- a notebook you carry with you.

Weight gain

You may gain weight when you are having this treatment. [Eating healthily and being active](#) can help you keep to a healthy weight. Your doctor, nurse or dietitian can give you more advice.

Raised blood sugar levels

This treatment can raise your blood sugar levels and increase the risk of diabetes.

Signs of raised blood sugar include:

- feeling thirsty
- needing to pass urine (pee) more often than usual
- feeling more tired than usual.

Tell your doctor or nurse if you have these symptoms. They can do tests to check your blood sugar level and give you advice.

If you already have [diabetes](#), your blood sugars may be higher than usual. You may need to check them more often, or make changes to your diabetic treatment. Your diabetic doctor or nurse will talk to you about how to manage this.

Effects on the liver

This treatment may affect how your liver works. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your liver is working.

Muscle or joint pain

You may get pain in your muscles or joints. If this happens, tell your doctor or nurse. They can give you advice and painkillers or other drugs to help. Being physically active and keeping to a healthy weight may help with the pain. It can keep your joints flexible and reduce stress on them. If the pain does not get better, tell your doctor or nurse. They may talk to you about changing to a different hormonal drug.

Allergic reaction

This treatment might cause an allergic reaction. But this is not common. Signs of a reaction can include:

- a skin rash or itching
- feeling breathless or wheezy
- swelling of your face, mouth or throat.

If you feel unwell or have any of these signs, contact the hospital straight away on the 24-hour number. Do not take any more of this treatment until you have checked with them.

Always call 999 if swelling happens suddenly or you are struggling to breathe.

Second cancer

This treatment can increase the risk of developing a second cancer years later. This is rare. The benefits of treatment usually far outweigh this risk. Your doctor, nurse or pharmacist can give you more information.

Effects on the brain

Rarely, this treatment causes a brain condition that can be serious. You can make a full recovery from this. But it must be diagnosed and treated quickly.

This condition can cause:

-

a headache that does not get better

- drowsiness or confusion
- changes in eyesight
- fits (seizures).

If you have any of these symptoms, it is important to either:

- contact the hospital straight away on the 24-hour number
- go to the hospital straight away.

You should not drive yourself to hospital.

Other important information

Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and [coronavirus \(covid\)](#). They may also recommend other vaccines, such as Shingrix[®] for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Contraception

Your doctor, nurse or pharmacist will advise you not to [make someone pregnant](#) while having this treatment. The drug may harm the developing baby. It is important to use effective contraception during treatment and for a while after it finishes.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

Date reviewed

Reviewed: 01 May 2026 | Next review: 01 May 2028

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help